
AFTER THE DEMO • FOR YOUR REVIEW

What you saw, and what we'd recommend.

A short, honest record of the demo — what it did on your own files, what it costs your staff in real terms, what happens when it doesn't work perfectly, and the smallest first step.

PREPARED FOR — TYPE B PRACTICE

Practices with an EHR, but a workflow stitched from disconnected tools.

You have an EHR. But billing, video, dictation and the queue don't talk to each other.

This document is written specifically for a practice in your situation. It is not a generic brochure — the recommendation in it is the one configuration we would actually put in your practice.

READ THIS
BEFORE YOU DECIDE.
IT IS DELIBERATELY SHORT.

You already run an EHR — so this was never about replacing the system you trust as your record. In the demo, we showed two things: your legacy archive becoming structured data alongside that EHR, and what it looks like when the surrounding workflow stops being four tools stitched together by hand.

— Your archive, structured — without touching your EHR

We took your own legacy files and extracted them into structured records, exportable straight into the EHR you already run. Your system of record did not change.

— The questions your EHR can't answer

Most EHRs store data well and analyse it poorly. We asked the kind of cross-practice question your current system has no screen for, and got an answer with the source attached.

— The stitching, gone

We showed what the day looks like when billing, notes, video and the queue are one flow rather than four browser tabs and a WhatsApp.

02 WHAT THIS ACTUALLY COSTS YOU IN STAFF TIME

The honest answer to the question you are really asking — “my reception is already drowning, what does this add to her day?”

Per document

30–60 seconds. The system extracts; your staff reviews what it found, corrects anything wrong, and approves. They know your handwriting; the system handles most of it; they catch the rest.

Week one

Scanning happens into the same folder your reception already scans into. The only new habit is the validation queue — a list of documents waiting for a 30-second check. That is the entire workflow change.

Month three

The queue is a background habit, not an event. The structured archive is large enough that the search and analytics start answering real questions. Nothing about the consult has changed.

What does not change

How you consult. What you write. Your existing system, if you have one. We are a layer that adds capability, not a system that takes work away from one that already works.

Demos show the happy path. This is the part that matters more: what the system does when a scan is bad, the handwriting is unreadable, or the extraction is unsure. We would rather you read this now than discover it later.

Low-confidence extraction

Every extracted field carries a confidence score. Low-confidence fields are flagged for the validator, not silently accepted. The human sees exactly what the system was unsure about.

An unreadable document

If a scan is too poor to extract, it does not get guessed at. It is held in the queue, flagged, and a person decides — rescan, enter by hand, or set aside. The system never invents data to fill a gap.

The wrong-patient safeguard

Nothing enters a patient's record until a person confirms it is the right patient. The match is shown; the human says yes or no. Wrong-patient errors are caught at that gate — before they happen, not discovered months later.

Every action is logged

Every approval, correction and reversal is recorded — who did it, when. If a medical aid audits the practice, or you need to show the basis of a record, the history is one query away.

04 WHY A SOUTH AFRICAN PRACTICE CHOOSES THIS

BUILT HERE, NOT BOLTED ON

Medical-aid schemes, HPCSA prescription rules, NAPPI codes, the SA edition of ICD-10, local payments — native to the system, not adapted to it after the fact.

IT WORKS WITH WHAT YOU ALREADY HAVE

Export and ingest via FHIR, CSV or API. You do not rip out a system that works to get the parts you don't have.

DIGITISATION AS A SUBSCRIPTION, NOT A PROJECT

The reason practices stay on paper is the six-figure project quote nobody signs. A predictable monthly page allowance instead — payable, cancellable, no capital sign-off.

EACH PRACTICE'S DATA IS ISOLATED

Multi-tenant by design. What happens at one practice never crosses to another.

Your EHR stays. This is explicitly not a rip-and-replace — you paid for that system and it holds your record. What you do not have is a workflow that hangs together, and a view of your own data. We address those two without disturbing the first.

The practice platform — Professional, as consolidation

Not to replace your EHR as the record, but to end the tool-stitching: AI Scribe drafting your notes from the consult, telehealth and e-scripts, reception and queue and vitals on one flow. The patchwork becomes one system.

Digitisation of the back room

Your legacy archive becomes structured data, exported back into the EHR you already run via FHIR or CSV. The archive stops being dead weight; your system of record stays exactly where it is.

The analytics view

Chronic-disease cohorts, claims aging, no-show patterns, productivity — ingested from your existing EHR. The analysis layer your current system was never built to be.

The honest framing: we are not telling you your EHR was the wrong choice. We are removing the four-tools-that-don't-talk tax you pay every evening, and giving you the view of your data the EHR was never designed to provide.

The same discipline we apply to the product, applied to this document.
These are real and they are not in scope of what you would be buying.

FINAL PRICING

Set per practice on doctor count, page volume and integration scope. The pilot price is fixed and stated; the full quote is a conversation, not a number on a leave-behind.

CLINICAL DECISION SUPPORT

Our clinical-AI work is an invite-only beta, not a commercial product, with no medical-device approval. It is a design-partner conversation if you raise it — it is deliberately not part of what this document offers you.

ROADMAP ITEMS

Medical-aid claims auto-submission, multi-site management and native EHR integrations are planned, not promised here. If it is not in the configuration above, treat it as not yet real.

THE SMALLEST FIRST STEP

One month. Your files. No lock-in.

Start with digitisation only — the lowest-risk module and the one that touches your EHR least. One month, 1,500 pages of your choosing, exported in the format your current EHR ingests. You see the extraction quality on your own documents and confirm the export lands cleanly in the system you already run, before any conversation about the wider workflow. Cancel any time, no setup fee, no lock-in.

Prognosis Labs

luzuko@medicdata.co.za · +27 (0) 76 969 5462

medicdata.co.za · [book a follow-up](#) — medicdata.co.za/demo